

obliterated, but a single line should be drawn through the word to be changed, and the correct information should be written above with the date of the change and the person's signature or legible initials. Further, an explanation as to why the record is being altered should be noted.

Good notes are of great value, not only when handing a patient over to another doctor, but also in meeting any criticism that may arise. If a patient refuses to accept the advice of his doctor, this fact should be recorded in writing. When there is a conflict of evidence, the Court will attach importance to the notes written at the time. Good notes may be of the greatest importance in supporting the doctor's evidence as against that of the plaintiff and his witnesses.

Patient has the right to know what is in his records and is entitled to a copy of his hospital record on discharge, by paying the cost of reproduction, but not to the original records. The next of kin can get the record in case of patient's death. Hospital has the responsibility to supervise the maintenance of appropriate, accurate, timely and up-to-date patient's records. The rights of patients to have their hospital medical records regarded as confidential must be respected. If in the doctor's judgement making the record available to the patient would be harmful or dangerous to the patient, or not in his best interest (professional or therapeutic discretion), the hospital can avoid to issue the record to the patient. The medical records of a patient (in-patient or out-patient) should not be given to any person without the consent of the patient. The police do not have a right to demand medical records except when there is statutory provision for such requisitions. The patient's record cannot be used in educational or diagnostic conferences or clinics or for publications, without the patient's consent. Hospitals have right to use the medical records without the consent of patient for statistical purposes and quality of care determinations. In the absence of agreement to the contrary, X-ray plates are the property of the treating doctor as part of his case record. The patient buys the skill and treatment rather than the X-ray films.

FORMAT FOR MEDICAL RECORD

(Regulation 3.1 of M.C.I.)

- (1) Name of the patient (2) Age
- (3) Sex (4) Address (5) Occupation
- (6) Date of first visit
- (7) Clinical note (summary) of the case
- (8) Provisional diagnosis
- (9) Investigations advised with reports
- (10) Diagnosis after investigation

(11) Advice (14) Observations

(12) Follow up

(13) Date

Signature in full

Name of treating physician

CONSENT IN MEDICAL PRACTICE

Consent means voluntary agreement, compliance or permission. Consent signifies acceptance by a person of the consequences of an act that is being carried out. To be legally valid, it must be given after understanding what it is given for, and of risks involved.

Reasons for obtaining Consent : (1) To examine, treat or operate upon a patient without consent is assault in law, even if it is beneficial and done in good faith. The patient may recover damages. (2) If a doctor fails to give the required information to patient before asking for his consent to a particular operation or treatment, he may be charged for negligence.

Kinds of Consent : Consent may be : (1) Express, i.e. specifically stated by the patient, or (2) Implied. Express consent may be (a) verbal, or (b) written.

Implied Consent: An adult patient of sound mind who (1) knows that he can either agree or refuse to submit to treatment or an operation, (2) knows or has been fully or fairly informed by his doctor as to what is to be done, and (3) then cooperated with the physician, has impliedly consented in words. The fact that a patient attends the hospital or calls the doctor to his house complaining of illness, implies that he consents to a general physical examination, to determine the nature of the illness. Consent is implied when a patient holds out his arm for an injection. Such implied consent is the consent usually given in routine practice.

Full Disclosure : The facts which a doctor must disclose depends on the normal practice in his community, and on the circumstances of the case. In general, the patient should ordinarily be told everything. The doctor has to decide, after taking into consideration all aspects of the patient's personality, physical and mental state, how much can be safely disclosed. The doctor need not disclose risks of which he himself is unaware. A physician need not inform the patient of risks that a person of average

intelligence would be aware of, or in an emergency situation. The physician need not give information to those patients who waive their rights, but the waiver should be clearly written in the record.

Therapeutic privilege: This is an exception to the rule of "full disclosure". Full disclosure of remote or theoretical risks involved could result in frightening a patient who is already fearful or who is an emotionally disturbed individual, and who may refuse the treatment when there is really little risk. It is only in the case of frank psychosis or extreme psycho-neurosis that the patient will be incapable of accepting the information. In these cases, the doctor may use discretion as to the facts which he discloses. The doctor should carefully note his decision in the patient's record, explaining his intentions and the reasons. He should request a consultation to establish that the patient is emotionally disturbed. The presence of a malignancy, or an unavoidable fatal lesion may not be disclosed, if the doctor feels the patient is not able to tolerate the knowledge. If possible, the physician should explain the risks to the patient's spouse or next of kin.

Prudent patient rule, i.e. what a prudent person in the patient's position would have decided if adequately informed about all the reasonably foreseeable risks.

INFORMED CONSENT : Informed consent implies an understanding by the patient of (1) the nature of his condition, (2) the nature of the proposed treatment or procedure, (3) expectations of the recommended treatment and the likelihood of success, (4) the details of the alternative courses of treatment that are available, (5) the risks and benefits involved in both the proposed and alternative procedure, (6) the potential risks of not receiving treatment, and the relative chances of success or failure of both procedures, (7) particular known inherent risks that are material to the informed decision, so that he may accept or reject the procedure. All disclosures must be in language the patient can understand. Physicians have a legal, moral and ethical duty to provide all relevant information that enables a patient to either accept or reject treatment. This disclosure will very much reduce litigation, when the results are unsatisfactory or unexpected. The patient must show that the doctor did not adhere to accepted medical

standards to prove liability for lack of informed consent.

Exceptions to informed consent : (1) Emergency. (2) Therapeutic privilege. (3) When a patient waives his right to informed consent and delegates the right to the doctor or a close relative.

Informed Refusal : The physician has a duty to disclose adequately and appropriately to the patient, the risks or possible consequences of refusal to undergo a test or treatment. After understanding all the facts, the patient can refuse to submit to treatment or an operation.

Examination may reveal findings which when used in the process of investigation can damage the party examined. If later on the party is proved to be innocent, the damage sustained cannot be undone. This is why the right to deny consent to examination is generally given to the party.

Paternalism is an abuse of medical knowledge so as to distort the doctor-patient relationship in such a way that the patient is deprived of his autonomy, or of his ability to make a rational choice. The doctor does not disclose the nature of the illness and the proposed treatment depriving the right of patient to accept or reject treatment.

RULES OF CONSENT : (1) Consent is necessary for every medical examination. Ordinarily, formal consent to medical examination is not required, because the patient behaves in a manner which implies consent.

(2) Oral consent should be obtained in the presence of a disinterested third party, e.g., nurse.

(3) **Written consent is not necessary in any case. However, it should be taken for proving the same in the Court if necessity arises.** Written consent should refer to one specific procedure, and not blanket permission on admission to hospital. Written consent should be in proper form and suitably drafted for the circumstances. The consent form should include specific consent to the administration of a general anaesthetic. The nature of the operation should be entered on the form as precisely as is consistent with the best interests of the patients. The wording should include a phrase to confirm that the patient has been informed of the nature of the procedure, before signing takes place. The written consent should be witnessed by another person, present at the signing to prevent

any allegation that the consent was forged or obtained under pressure or compulsion.

(4) Any procedure beyond routine physical examination, such as operation, blood transfusion, collection of blood, etc. requires express consent. It must be taken before the act, but not at the time of admission into the hospital. It should be ongoing process extending overtime.

(5) The doctor should explain the object of the examination to the patient, and patient should be informed that the findings will be included in a medical report.

(6) In medicolegal cases the doctor should inform the patient that he has right to refuse to submit to examination and that the result may go against him. If the patient refuses, he cannot be examined.

(7) **The consent should be free, voluntary, clear, intelligent, informed, direct, and personal.** There should be no fraud, misrepresentation of facts, undue influence, compulsion, threat of physical injury, death or other consequences.

(8) **In criminal cases, the victim cannot be examined without his/her consent. The Court also cannot force a person to get medically examined, against his will.** (A) In cases of rape, the victim should not be examined without written consent. (B) In medicolegal cases of pregnancy, delivery and abortion, the woman should not be examined without her consent.

(9) **S.53, Cr.P.C.** (1) A person is arrested on a charge of committing an offence, and there may be reasons for believing that an examination of his person will provide evidence as to the commission of an offence. A registered medical practitioner can examine such person, even by using reasonable force, if the examination is requested by a police officer not below the rank of sub-inspector (or any other officer acting under his direction and good faith). "Examination" includes, examination of blood, blood stains, semen, swabs in case of sexual offences, sputum, sweat, hair samples and fingernail clippings by the use of modern and scientific techniques including DNA profiling and such other tests that a medical practitioner thinks necessary in that particular case. If the accused refuses examination, this may go against him in criminal proceedings. (2) In the case of

a female, the examination should be made only by or under the supervision of a female registered medical practitioner.

S.54, Cr.P.C. An arrested person at his request may be examined by a doctor to detect evidence in his favour. (2) A copy of the report of such examination is to be furnished by the medical practitioner to the arrested person or the person nominated by such arrested person.

(10) In cases of drunkenness, the person should not be examined and blood, urine, or breath should not be collected without his written consent. But, if the person becomes unconscious or incapable of giving consent, examination and treatment can be carried out. The consent of guardian or of relatives if available, should be taken. **The person can be examined without consent, if requested by the sub-inspector of police.**

(11) **Sec.87, I.P.C.** A person above 18 years of age can give valid consent to suffer any harm, which may result from an act not intended or not known to cause death or grievous hurt.

A person may be suffering from a disease which is certain to shorten his life. He can give free and informed consent to take the risk of operation, which though fatal in the majority of cases is the only available treatment. The surgeon cannot be held responsible, if the patient dies.

(12) **Sec. 88, I.P.C.** A person can give valid consent to suffer any harm which may result from an act, not intended or not known to cause death, done in good faith and for its benefit.

If a surgeon operates on a patient in good faith and for his benefit, even though the operation is a risk, he cannot be held responsible if the patient dies.

S. 88, I.P.C. is ambiguous and is not specific for medical treatment. The implication is that consent of parents or guardians is necessary for surgical or medical procedures if the patient is a minor.

(13) **Sec.89, I.P.C.** A child under 12 years of age and an insane person cannot give valid consent to suffer any harm which may result from an act done in good faith and for its benefit. The consent of the parent or guardian should be taken. If they refuse, the doctor cannot treat the patient even to save the life.

A father giving consent for an operation on the child in good faith and for the child's benefit, even though the operation is risky, cannot be held responsible if the child dies.

Loco Parentis : In an emergency involving children, when their parents or guardians are not available, consent is taken from the person-in-charge of the child, e.g., a school teacher can give consent for treating a child who becomes sick during a picnic away from home town, or the consent of the headmaster of a residential school.

(14) **Sec. 90, I.P.C.** A consent given by a person under fear of injury, or due to misunderstanding of a fact is not valid. The consent given by an insane or intoxicated person, who is unable to understand the nature and consequences of that to which he gives his consent is invalid.

To represent to a patient that an operation is necessary to save life or to preserve health when that is not the case or to indicate that it will give greater relief than there is any reasonable prospect of obtaining is to perpetrate a fraud on the patient that vitiates his consent.

(15) **S.52, I.P.C.** Any harm caused to a person in good faith, even without that person's consent is not an offence, if the circumstances are such, that it is impossible for that person to signify consent, and has no guardian or other person in lawful charge of him from whom it is possible to obtain consent in time for the thing to be done in benefit (Sec. 92, I.P.C.). Nothing is said to be done in good faith which is done without due care and attention.

A person may be involved in an accident, which may necessitate an amputation; if it is done without his consent, it is not an offence. In an emergency, the law implies consent. An emergency is defined as a medical situation, such as to render immediate treatment advisable either to save life or to safeguard health.

In an emergency, a comatose patient requiring immediate treatment, a mentally incompetent patient requiring treatment when a legal guardian is not available, an intoxicated patient who temporarily lacks the capacity to consent but requires treatment, consent is implied. A doctor may extend a procedure beyond the scope of consent to treat an emergency.

(16) **S.93, I.P.C.** Any communication made in good faith for the benefit of a person is not an offence, if it causes harm to that person to whom it is made.

A physician in good faith tells a patient that he cannot live. The patient dies in consequence of the shock. The physician has not committed any offence.

(17) The doctor should inform reasonably to the patient about the nature, consequences and risks of the examination or operation before taking the consent.

In an obscure case, the doctor should obtain an open consent to use his discretion for additional procedures needed to cope with unanticipated situations that endanger his health (**extension doctrine**). It is not applicable to non-essential procedures. When there are two or more methods of treatment, the patient should be allowed to choose and give consent for any method.

If in the course of an operation to which the patient has consented, the physician discovers conditions that had not been anticipated before the operation began, and which would endanger the life or health of the patient if not corrected, the doctor would be justified in extending the operation to correct them, even though no express consent was obtained. If an anaesthetist administers a type of anaesthetic expressly prohibited by the patient, he will be responsible for damages resulting from an unfortunate occurrence caused by the anaesthetic, even though there is no negligence in its administration.

(18) Consent of the inmates of the hostel, etc., is necessary if they are above 12 years. Within 12 years, the head master or warden can give consent. If an inmate above 12 years refuses treatment, and he is likely to spread the disease, he can be asked to leave the hostel. However, if he stays in hostel, he can be treated without his consent.

(19) When an operation is made compulsory by law, e.g., vaccination, the law provides the consent.

(20) A prisoner can be treated forcibly without consent in the interest of the society.

(21) Consent given for committing a crime or an illegal act, such as criminal abortion is invalid.

(22) Consent is not a defence in cases of professional negligence.

(23) The nature of illness of a patient should not be disclosed to any third party without the consent of the patient.

(24) For contraceptive sterilisation, consent of both the husband and wife should be obtained.

(25) The consent of one spouse is not necessary for an operation or treatment of other. A husband has no right to refuse consent to any operation, including a gynaecological operation, which is required to safeguard the health of his wife. The consent of wife is enough. It is advisable to take the consent of the spouse whenever practicable, especially if the operation involves danger to life, may destroy or limit sexual functions, or may result in the death of an unborn child.

(26) It is unlawful to detain an adult patient in hospital against his will. If a patient demands discharge against medical advice, this should be recorded and his signature obtained.

(27) A living adult person can give consent for donating one of his kidneys to be grafted into another person. The donor must be informed of the procedure involved and possible risks. The donation should not be accepted, if there is any risk of life of donor.

(28) If any person has donated his eyes to be used for therapeutic purpose after his death, the eyes can be removed only with the consent of guardian or legal heirs.

(29) **If any person has donated his body to be used for therapeutic or research purposes after his death, it is not binding on his spouse or next of kin.** For organ transplantation, the organs of the dead person, such as heart, kidney, liver, etc. should not be removed without the consent of the guardian or legal heirs. Precautions should be taken to preserve the anonymity of both donor and recipient.

(30) **Pathological autopsy should not be conducted without the consent of the guardian or legal heirs of the deceased.** If the autopsy is done without consent, the doctor is liable for damages for the mental anguish suffered by heirs due to the mutilation of the body. Specific authorisation should be obtained for retention of organs and parts of the body. **In medicolegal autopsies (statutory authorisation), consent is not required and the doctor can remove from the cadaver anything that is essential for purposes of examination.**

CASE : (1) MOSS V. RISHWORTH: A 11 year old girl was taken to surgeon for removal of tonsils and adenoids by her two adult sisters. The child died under anaesthetic. The Court held that there was no emergency which would excuse the need for parental consent, and the father could recover damages.

(2) **JOCKOVACH V. YOCUM:** The arm of a 7 year old boy was crushed by a train. The boy's arm was amputated immediately as the doctors could not contact parents. The consent of the parents was implied by the emergency.

(3) **WELLS V. MC GEHEE:** A 7 year old child died under anaesthesia for treatment of a broken arm, which was given without the consent of the mother as she could not be contacted. The Court held that an emergency existed.

(4) **DRUMMOND'S CASE:** Drummond sued a woman patient for recovery of fees. The patient counterclaimed damages as a drug was administered to her, without her consent. She alleged that phenobarbitone, which she refused to take, was mixed in soup and meat and given to her daily, which prolonged her stay in the nursing home, as a psychological consequence for 16 weeks. The Court held that the administration of a drug to a person without that person's knowledge and consent was assault, and awarded nominal damages as the drug did not cause substantial harm.

EUTHANASIA (mercy killing): It means producing painless death of a person suffering from hopelessly incurable and painful disease.

Types: (1) Active or positive. (2) Passive or negative. **Active euthanasia** is a positive merciful act, to end useless suffering or a meaningless existence. It is an act of commission, e.g. by giving large doses of drugs to hasten death. **Passive euthanasia** means discontinuing or not using extraordinary life-sustaining measures to prolong life. This includes acts of omission, such as failure to resuscitate a terminally ill or hopelessly incapacitated patient or a severely defective newborn infant. It is not using measures that would probably delay death such as, turning off a respirator, stopping medications or food and water and allowing the person to dehydrate or starve or not delivering (cardio-pulmonary resuscitation), which permits natural death to occur.

Voluntary euthanasia means at the will of the person, and **involuntary** means against the will of the person, i.e., compulsory. **Non-voluntary** refers to cases of persons incapable of making their wishes known, e.g., in persons with irreversible coma or severely defective infants. Euthanasia advocates the administration of lethal doses of opium or other narcotic drugs. It has no legal sanction.

Assisted Suicide : A person providing information to another with information, guidance and means to take his own life with the intention that it will be used for this purpose is assisted suicide

Terminal sedation: It includes the administration of morphine and similar medications, which has a dual effect of relieving of pain and hastening the death (**aid-in-dying**). If the patient requests the same medical treatment with its known dual effects, and if the physician knowingly provides that medication

by prescription so that patient can end his life, it is considered **physician-assisted suicide**.

Article 21 of the constitution of India guarantees "right to live", but does not imply "right to die".

The Netherlands was the first country to legalise passive voluntary euthanasia and assisted suicide in 2002, if the following conditions were met: (1) The disease is incurable. (2) The patient's suffering is unbearable. (3) Patient's condition is terminal. (4) Patient requests death. Another physician must be consulted first and life must be ended in a medically appropriate way. Other countries which have joined this select group are Belgium, Luxemburg, Switzerland, Thailand.

Iatrogenic Diseases: Iatrogenic disease can be defined as a disease that results from administration of a drug, or medical or surgical acts for prophylaxis, diagnosis or treatment.

DEATHS DUE TO MEDICAL CARE: Deaths may occur due to : (1) Complications of anaesthesia. (2) Complications of surgery. (3) Nosocomial infections. The use and/or misuse of urinary catheters, techniques and equipment employed in intravenous therapy; hyperalimentation, and respiratory therapy cause most of these infections. (4) Therapeutic misadventure. (5) Professional negligence. (a) Administration of wrong dose. (b) Pharmacist dispensing wrong medicine due to illegible prescription. (c) Abbreviations of drugs. (d) Confusing patients with similar names and to administer the correct dose to the wrong patient. (e) Susceptibility of children to medication errors. (f) New approved drugs may result in death from side-effects undetected in the study population. (g) Patient or specimen misidentification in laboratory testing can lead to inappropriate and potentially life-threatening therapy. (h) Errors in reporting abnormal values in electrolyte analysis. (i) Mismatched blood transfusion.

PENAL PROVISIONS APPLICABLE TO MEDICAL PRACTICE:

S. 52, I.P.C. : Nothing is said to be done in good faith which is done without due care and attention.

S. 74, I.P.C. : Nonattendance in obedience to summons from court (6 months imprisonment).

S. 118, I.P.C. : Concealing design to commit offence punishable with death or imprisonment for life. (imprisonment for 3 months if the offence is not committed and 7 years if the offence is committed).

S. 175, I.P.C. : Omission to produce document to public servant by person legally bound to produce it (six months imprisonment).

S. 176, I.P.C. : Omission to give notice or information to public servant by person legally bound to give it (imprisonment up to one month).

S. 177, I.P.C. : Furnishing false information (imprisonment up to six months).

S. 178, I.P.C. : Refusing oath or affirmation when duly required by public servant to make it (imprisonment up to six months).

S. 179, I.P.C.: Refusing to answer public servant authorised to question (imprisonment up to six months).

S. 182, I.P.C. : False information with intent to cause public servant to use his lawful power to the injury of another person (imprisonment up to six months).

S. 191, I.P.C. : Giving false evidence.

S. 192, I.P.C. : Fabricating false evidence.

S. 193, I.P.C. : Punishment for false evidence (imprisonment up to seven years).

S. 194, I.P.C. : Giving or fabricating false evidence with intent to procure conviction of capital offence (imprisonment up to ten years).

S. 195, I.P.C. : Giving or fabricating false evidence with intent to procure conviction of offence punishable with imprisonment for life or imprisonment.

S. 197, I.P.C. : Issuing or signing false certificate (imprisonment up to seven years).

S. 201, I.P.C. : Causing disappearance of evidence of offence, or giving false information to screen offenders (imprisonment up to ten years).

S. 202, I.P.C. : Intentional omission to give information of offence by person bound to inform (imprisonment up to six months).

S. 203, I.P.C. : Giving false information respecting an offence committed (imprisonment up to two years).

S. 204, I.P.C. : Destruction of document to prevent its production as evidence (imprisonment up to two years).

S. 269, I.P.C.: Negligent act likely to spread infection of disease dangerous to life (imprisonment up to six months).

S. 270, I.P.C.: Malignant act likely to spread infection of disease dangerous to life (imprisonment up to two years).

S. 284, I.P.C.: Negligent conduct with respect to poisonous substances (imprisonment upto 6 months).

S. 39, Cr. P.C. : Every person, aware of the commission of, or of the intention of any other person to commit any offence punishable under I.P.C. shall forthwith give information to the nearest Magistrate or police officer of such commission or intention.

S. 160, Cr.P.C. : Police officer has the power to summon any witness (doctor) to police station for recording a statement.

S. 161, Cr.P.C. : The police has the power to examine witnesses.

S. 162, Cr.P.C. : Oral statements made to the police and recorded by the police should not be signed.

Legal protection to medical doctors is provided by S. 87 to 93, I.P.C.

MALINGERING : Malingering or shamming means conscious, planned feigning or pretending a disease for the sake of gain. Diseases may be feigned for several reasons, such as by soldiers or policemen to avoid their duties, by prisoners to avoid hard work, by businessmen to avoid business contracts, by workmen to claim compensation, by beggars to attract public sympathy, by criminals to avoid legal responsibility, etc. The diseases that may be feigned are many, e.g., dyspepsia, intestinal colic, ulcers, spitting of blood, ophthalmia, diabetes, rheumatism, lumbago, neurasthenia, aphasia, sciatica, pain in the back, blindness, deafness, vertigo, epilepsy, insanity, paralysis of the limbs, burns, artificial bruises, etc. **Patients can distort or exaggerate their symptoms but true simulation is very rare.**

(1) The patient may injure his nasopharynx with a sharp instrument, swallow the blood and regurgitate it in front of the doctor to mimic haematemesis. (2) A skilful puncturing of the anal or vaginal mucosa, may produce bleeding. (3) Excessive intake of digitalis may simulate a heart condition. (4) Eating of large amount of carrot will produce carotinaemia and may simulate jaundice. (5) Chronic ingestion of coumarin will induce a haemorrhagic diathesis. In many cases detection is easy, but in some cases it is difficult.

The history of the case should be taken from the person himself, and his relatives or friends, and any inconsistencies in his description of the symptoms noted. Usually, the signs and symptoms do not conform to any known disease. **Malingering can be diagnosed by keeping the patient under observation and watching him without his knowledge.** A complete examination is essential after removing the bandages if any, and washing the part. Rarely an anaesthetic may be given to detect malingering.